



Dire Dawa University
STUDENT CLINIC CENTER

Version 2.0

— ARCHITECTURE & PRODUCT DOCUMENTATION

DDU Clinic — Platform Architecture & Product Documentation

A cloud-native, serverless, modular service-layered clinic management platform for the Dire Dawa University Student Clinic Center — governed by a declarative finite-state-machine engine, backend-driven affordances, and tamper-evident document verification.

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01 Executive Summary

The DDU Clinic platform is a from-scratch, cloud-native rebuild of the university's clinic management system — engineered so that process, security, and audit are one and the same act.

The **DDU Clinic platform** (package `ddu-clinic`, version `2.0.0`) is the ERP-style clinic management system for the **Dire Dawa University Student Clinic Center**. It is a from-scratch **Next.js 16 / React 19 / TypeScript** rebuild that runs **cloud-native and serverless** on Vercel against a **Neon serverless PostgreSQL** database, accessed in production through a pooled connection.

Architecturally, the system is best described — honestly — as a **modular, service-layered monolith that is decomposable**: a single Next.js application whose modularity comes from a disciplined domain-service layer (`src/server/services/*`), one file per bounded context, each owning its own writes and its own state machine. It is *not* a set of running microservices, but the boundaries are already drawn so any context could be peeled off later with limited blast radius.

8

Staff roles governed by RBAC, from Manager to Store Keeper

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Declarative state machines drive every clinical & operational entity

~35

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Signed document types with public HMAC verification

1

Source of truth for who-may-do-what: the FSM table

1.1 What sets it apart

Three ideas make the platform distinctive and are woven through every module:

- **A declarative FSM engine** is the single authority for which state transitions are legal and which role may fire each one. Services never change a `state` column without first passing through the engine's validation gate.
- **Backend-driven affordances (HATEOAS-style)**: the server computes the exact set of actions a given user may take on a given entity right now, and the client renders precisely those buttons — nothing more, nothing less. The UI can never drift from what the server will accept.
- **Tamper-evident documents**: every issued certificate carries a short public verify code and an HMAC-SHA256 signature over its immutable facts. A public `/verify` page recomputes the signature — a genuine document always verifies; a forged one never will.

HONESTY NOTE

This document deliberately avoids overclaiming. The platform is a **cloud-native, serverless, modular service-layered** application — not literal microservices. The PWA is **installable** via a web manifest but is **not** yet an offline-caching service worker (Serwist is a dependency, not yet wired). Every claim in this document is traceable to source on branch `feature/platform-v2`.

02 Product Overview

A campus clinic serving thousands of students and staff runs on registration desks, triage rooms, consultation, a lab, a pharmacy, wards, referrals, HR and a store — the platform ties all of it into one governed spine.

2.1 The problem

A university clinic at the scale of a **5,000–10,000-student campus** is a small hospital in disguise. On any given day it registers walk-ins, triages vitals, runs consultations, orders and results lab tests, dispenses medicines from expiring batches, admits and discharges inpatients, refers complex cases to external hospitals, bills for services, manages staff leave and attendance, and keeps a store of assets and supplies. Done on paper or across disconnected spreadsheets, this fragments the patient record, loses the audit trail, and makes it impossible to know — at any moment — *who is responsible for what right now*.

The specific failure modes the platform is designed to eliminate: a nurse and a receptionist disagreeing about whether a patient has been sent to the doctor; a lab result that never makes it back to the ordering physician; a medicine dispensed from a batch that expires next month while an older batch sits on the shelf; a sick-leave certificate that a student photoshops; a staff member on approved leave who can still log in and act.

2.2 The solution

The platform gives every clinical and operational process a **single, role-aware state machine**. Each hand-off — reception to triage, triage to doctor, doctor to lab, lab back to doctor, doctor to pharmacy — is a guarded transition that is validated on the server, wrapped in a database transaction, written to an audit trail, and fanned out as a notification to the next team, all in one atomic act. The workflow, the audit record, and the hand-off notification are the same operation.

DESIGN PRINCIPLE

Because the buttons a user sees and the transitions the server accepts are both projected from one declarative table, the interface can never offer an action the backend would refuse — and gating or adding an action is a one-line change to a machine definition.

2.3 Who uses it — the eight roles

Access is governed by role-based access control across eight staff roles, plus a self-service student portal.

MANAGER is always privileged and passes every role guard.

ROLE	PRIMARY RESPONSIBILITY	HOME SURFACES
MANAGER	Oversight, configuration, reports, feature flags, billing voids; always passes role guards.	Reports, Settings, Billing

ROLE	PRIMARY RESPONSIBILITY	HOME SURFACES
RECEPTIONIST	Front-desk registration and visit intake; billing and payments; referral acknowledgement.	Reception, Billing
NURSE	Triage and vitals capture; ward settling and discharge; admissions.	Triage, Wards
DOCTOR	Consultations: order labs, prescribe, refer, admit, complete; Saba clinical copilot.	Consultations, Patients
LAB_TECH	Lab worklist: collect, process, submit results back to the ordering doctor.	Laboratory
PHARMACIST	Dispensing queue and FEFO stock; batch/expiry inventory; suppliers.	Pharmacy, Inventory
HR	Leave requests and approvals (drives the on-leave lockout), attendance, rosters.	Human Resources
STORE_KEEPER	Non-medical inventory and assets; stock requests; purchase orders and goods receipt.	Store & Assets
STUDENT	Self-service portal: own appointments, documents, visit history, and the Saba health guide.	Student Portal

Roles from `Role` enum (`schema.prisma:22`); labels in `src/lib/rbac.ts`. Students authenticate separately and are confined to `/portal`.

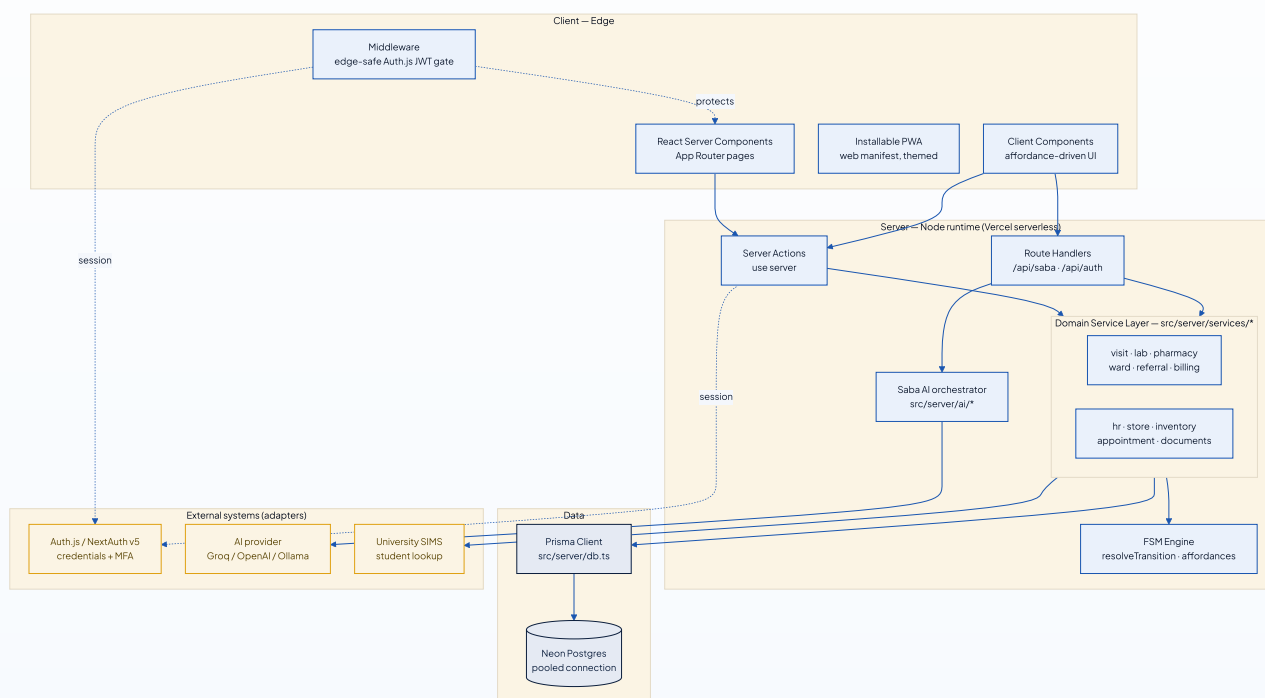
03 System Architecture

A layered, cloud-native architecture: React Server Components and an installable PWA at the edge, server actions and route handlers, domain services fronting the FSM engine, Prisma, and Neon Postgres — with a handful of external systems on clean adapters.

The platform follows a strict **request-inward** flow. The client renders Server Components and Server-Action forms; those call **into** the domain service layer; services are the only code that writes, and every write that changes an entity's state passes first through the **FSM engine**. Persistence is via Prisma to Neon Postgres. External systems — Auth.js, an OpenAI-compatible AI provider for Saba, and the university SIMS — are reached through narrow adapters, never inline.

FIG. 1 Layered system architecture

Client (RSC + installable PWA) → server actions / route handlers → domain services (FSM engine) → Prisma → Neon Postgres, with external systems on adapters.



3.1 The edge / Node runtime split

Authentication is deliberately split for the edge. `src/auth.config.ts` is an **edge-safe** configuration with no Prisma or Node APIs and empty `providers`; it is used by `src/middleware.ts` to do cheap JWT-based route protection at the edge without dragging Prisma into the edge bundle. The full `src/auth.ts` — credential providers, bcrypt, Prisma — runs on the **Node runtime**. Routes that need Node opt in explicitly: the Saba route sets `export const runtime = "nodejs"` and `dynamic = "force-dynamic"`.

3.2 The service layer is the seam

Every module is backed by a service in `src/server/services/*` — `visit`, `lab`, `pharmacy`, `ward`, `referral`, `billing`, `hr`, `inventory`, `store`, `appointment`, `clinical`, `patient`, `documents`, `document-signing`, `notifications`, `events`, `mfa`, `webauthn`, `settings` — plus `src/server/fsm/*` (state authority), `src/server/ai/*` (Saba), and `src/server/integrations/*` (SIMS). UI and API routes call into these services; the services own all writes and all FSM gating. Because each is a clean module with an explicit interface and its own state machine, a bounded context could later become its own deployable unit.

WHY THIS MATTERS

The disciplined service boundary is what makes the "modular monolith that is decomposable" claim real rather than aspirational — the lines along which the system could split are already the lines along which it is built.

04 The Three Architectural Pillars

One declarative table decides which transitions are legal and who may fire them; the same table renders the UI; and a feature-flag registry decides which modules are even present. Together they make drift between UI and enforcement structurally impossible.

4.1 Pillar I — The FSM engine

The finite-state-machine engine lives in `src/server/fsm/engine.ts`, with machine definitions in `src/server/fsm/machines/*.ts` and an XState mirror in `src/server/fsm/xstate.ts`. It is a **tiny declarative engine** — not raw XState at runtime. Each domain entity is described by a `MachineDef`: a table of states plus role-guarded transitions. A single `TransitionDef` carries the event name, one or more `from` states, a `to` destination, the `roles` allowed to fire it (omitted $\Rightarrow$ any authenticated user), UI hints (`label`, `description`, `intent`), an optional `form` key naming data the client must collect first, and an optional runtime `guard`.

Three exported functions

- `availableTransitions()` — filters by state, then role, then guard; returns a serializable `Affordance[]` shipped to the client.
- `canFire()` — boolean: can this actor fire this event now? (delegates to the above).
- `resolveTransition()` — the **validation gate** services call before persisting; throws a typed `FsmError` (`INVALID_STATE`, `ILLEGAL_TRANSITION`, `FORBIDDEN`).

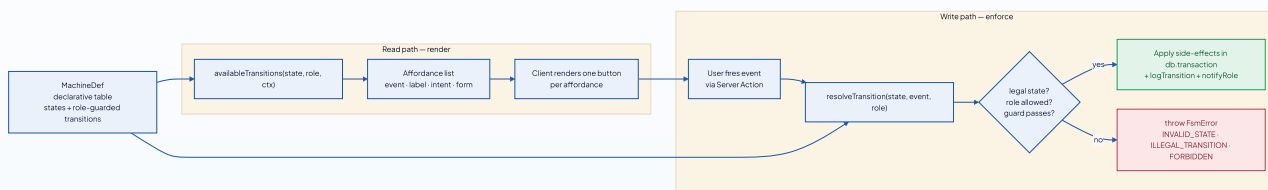
Single source of truth

The declarative table is the one authority for *which* transitions are legal and *who* may fire them. Services never mutate a `state` column without first calling `resolveTransition`. The same table drives the UI, so the buttons a user sees and the transitions the server accepts are guaranteed to agree.

Mirrored to XState: `toXStateMachine()` compiles each `MachineDef` into a real XState v5 statechart for visualization (Stately) and cross-checking in tests — the two stay lock-step.

FIG. 2 The FSM / affordance concept — one table, two consumers

The same declarative machine table both projects the affordance list the client renders and gates the write the server accepts, so the two can never diverge.



Transition execution & audit. Every state change flows through a service (e.g. `transitionVisit`): load the entity, run the `resolveTransition` gate, apply event side-effects inside a `db.$transaction` (create the lab/drug order, mark a bed occupied, set `closedAt`), update the `state` column, write a `StateTransition` row, notify the next team, and revalidate. The `StateTransition` model is a generic platform-wide event log keyed by `entityType` / `entityId`.

4.2 Pillar II — Backend-driven affordances (HATEOAS-style)

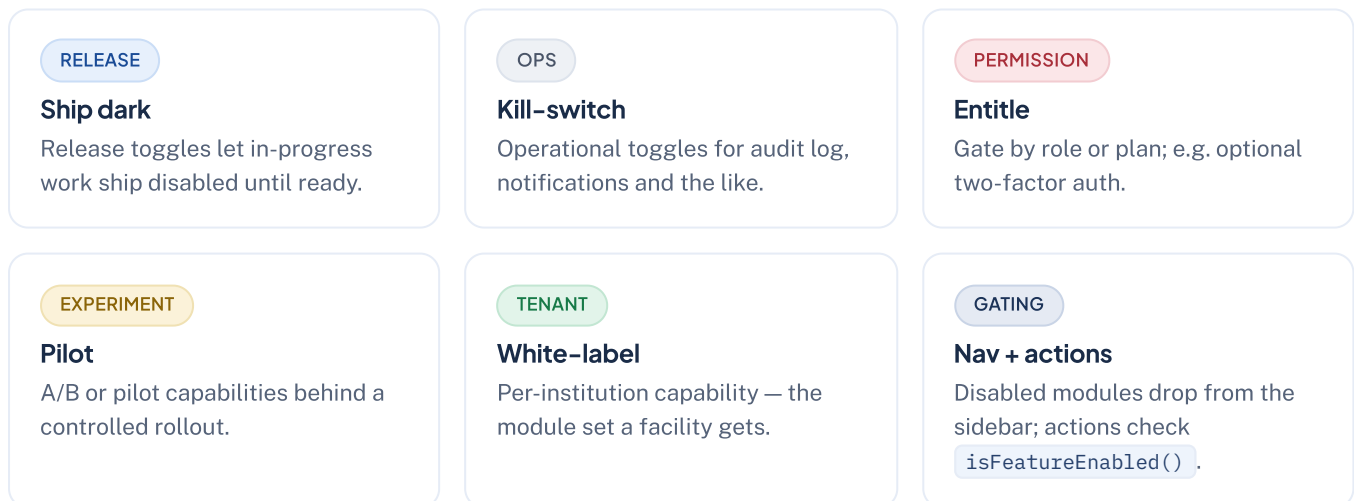
This is a core architectural idea: **the server decides what the client may do, and the client renders exactly that**. Verified in code, the flow is:

1. A Server Component page loads the entity and computes affordances for the current actor. For a visit, `visits/[id]/page.tsx` calls `visitAffordances(visit.state, actor)`, a thin wrapper over `availableTransitions(visitMachine, state, { role })`.
2. The page passes that `Affordance[]` into a client component — `<VisitActions affordances={...} />`.
3. The client renders **one button per affordance**, styling it from `affordance.intent` (`primary/success/danger` → button variants) and labelling it from `affordance.label`. If the affordance carries a `form` key, clicking opens the matching dialog to collect that data first.
4. Firing calls a Server Action which re-enters the service → `resolveTransition` re-validates the *same* rule server-side. The client is never trusted; the affordance list is a convenience projection of an authority that is re-checked on write.

The same pattern powers appointments, wards, referrals, lab orders, HR leave, billing, and store requests. Roles, legal transitions, button labels, styling intent, and required forms are all declared *once* in the FSM table — so the server enforcement can't drift from what the UI offers.

4.3 Pillar III — The feature-flag system

The registry `src/lib/features.ts` is a **Fowler-style toggle registry** and the source of truth: `FEATURES: FeatureDef[]` lists every flag with `key`, `name`, `description`, `category`, `defaultEnabled`, and `group`. DB `FeatureFlag` rows mirror the registry so a manager can flip toggles at runtime, and `rolloutRoles` supports PERMISSION-style role gating.



Resolution follows a clear precedence: `isFeatureEnabled(key, role?)` lets the DB row win, else the registry default, honoring `rolloutRoles` (MANAGER always passes). `enabledFeatureSet(role?)` yields the enabled keys used to filter nav and UI; `setFeatureFlag()` upserts and revalidates the layout; `syncFeatureRegistry()` idempotently upserts all registry flags into the DB. Nav items carry an optional `feature` key, so disabled modules simply vanish from the sidebar.

05 Modules & Feature Inventory

Roughly thirty-five feature flags across six groups, plus a module-by-module map of the whole platform. Categories carry intent — tenant capability, release toggle, operational switch, or permission gate.

Default ON

Default OFF

TENANTwhite-label

RELEASE

OPS

PERMISSION

5.1 Modules — tenant capabilities (all TENANT)

KEY	NAME	DESCRIPTION	DEFAULT
module.reception	Reception & registration	Front-desk registration and visit intake.	ON
module.triage	Nurse triage	Vitals capture and triage queue.	ON
module.doctor	Consultations	Doctor consultation workspace.	ON
module.lab	Laboratory	Lab orders, worklist and results.	ON
module.pharmacy	Pharmacy	Dispensing and medicine inventory.	ON
module.wards	Wards & admissions	Inpatient beds and admissions.	ON
module.referrals	Referrals	External hospital referrals.	ON
module.hr	Human resources	Staff, leave, attendance & payroll.	ON
module.store	Store & assets	Non-medical inventory and assets.	ON
module.reports	Reports & analytics	Management dashboards.	ON
module.portal	Student portal	Student self-service portal.	ON
module.billing	Billing & cashier	Charges, invoices and payments.	ON
module.appointments	Appointments	Scheduling and calendar.	ON

5.2 Clinical, Documents, HR & Store, Platform

KEY	NAME	CATEGORY	DEFAULT
CLINICAL			
clinical.sims_lookup	SIMS student lookup	TENANT	ON
clinical.allergies	Allergy list	RELEASE	ON

KEY	NAME	CATEGORY	DEFAULT
<code>clinical.problem_list</code>	Problem list	RELEASE	ON
<code>clinical.vitals_trends</code>	Vitals trends	RELEASE	ON
<code>clinical.immunization</code>	Immunizations	RELEASE	ON
<code>clinical.icd10</code>	ICD-10 coding	RELEASE	ON
<code>clinical.consent</code>	Consent capture	RELEASE	OFF
DOCUMENTS — all TENANT			
<code>docs.sick_leave</code>	Sick-leave certificates	TENANT	ON
<code>docs.prescription_print</code>	Prescription printout	TENANT	ON
<code>docs.referral_print</code>	Referral letter	TENANT	ON
<code>docs.lab_report_print</code>	Lab report printout	TENANT	ON
HR & STORE			
<code>hr.attendance</code>	Attendance & shifts	RELEASE	ON
<code>hr.payroll</code>	Payroll	RELEASE	OFF
<code>hr.leave_balances</code>	Leave balances	RELEASE	ON
<code>store.purchase_orders</code>	Purchase orders	RELEASE	ON
<code>store.stock_counts</code>	Stock counts	RELEASE	ON
PLATFORM / UX			
<code>ux.command_palette</code>	Command palette (⌘K)	RELEASE	ON
<code>ux.dark_mode</code>	Dark mode	RELEASE	ON
<code>ux.notifications</code>	Notifications centre	OPS	ON
<code>platform.audit_log</code>	Audit log	OPS	ON
<code>platform.two_factor</code>	Two-factor auth	PERMISSION	OFF
<code>ai.assistant</code>	Saba AI assistant	RELEASE	OFF
<code>platform.white_label</code>	White-label branding	TENANT	ON

Notable defaults-off: `clinical.consent`, `hr.payroll`, `platform.two_factor`, `ai.assistant`. Everything else defaults on. Verified in `src/lib/features.ts`.

5.3 Module-by-module map

App routes live under `src/app/(app)/**` (staff shell), `src/app/portal/**` (student portal), and `src/app/print/**` (printables). Access rules are in `SECTION_ACCESS` (`src/lib/rbac.ts`); each module is backed by a service.



CLINICAL GROUP

From the front desk to discharge

Reception, triage, consultations, lab, pharmacy, wards, referrals and the live queue board.

MODULE	ROUTES	ROLES
Dashboard	<code>/dashboard</code> — role-aware landing	<code>any staff</code>
Reception	<code>/reception</code> — registration, creates <code>REGISTERED</code> visits	RECEPTIONIST · MANAGER
Patients	<code>/patients</code> , <code>/[id]</code> , vitals, allergies/problems/immunizations	RECEPTIONIST · NURSE · DOCTOR · MANAGER
Appointments	<code>/appointments</code> , <code>/new</code> , <code>/[id]</code> — scheduling FSM	RECEPTIONIST · NURSE · DOCTOR
Triage	<code>/triage</code> — vitals capture, triage queue	NURSE · MANAGER
Consultations	<code>/doctor</code> , <code>/doctor/caseload</code> , <code>/visits/[id]</code> — Saba copilot attaches here	DOCTOR · MANAGER
Laboratory	<code>/lab</code> , <code>/lab/[id]</code> , <code>/lab/catalog</code>	LAB_TECH · MANAGER
Pharmacy	<code>/pharmacy</code> , <code>/[id]</code> , <code>/inventory</code> , <code>/suppliers</code>	PHARMACIST · MANAGER
Wards	<code>/wards</code> , <code>/wards/[id]</code> — admission FSM	NURSE · DOCTOR · MANAGER
Referrals	<code>/referrals</code> , <code>/new</code> , <code>/[id]</code> — signed printable letter	DOCTOR · RECEPTIONIST · MANAGER
Queue board	<code>/queue</code> — live waiting-room display	<code>any staff</code>



OPERATIONS & MANAGEMENT

Billing, HR, store and oversight

The back-office spine — cashier, human resources, procurement, staff directory, reports and settings.

MODULE	ROUTES	ROLES
Billing	<code>/billing</code> , <code>/billing/[id]</code> — invoice FSM + printable receipt	RECEPTIONIST · MANAGER

MODULE	ROUTES	ROLES
Human Resources	/hr , /hr/leave , /attendance , /leave-balances , /roster	HR · MANAGER
Store & Assets	/store , /assets , /requests , /purchase-orders	STORE_KEEPER · MANAGER
Staff Directory	/staff , /staff/[id]	MANAGER · HR
Reports	/reports — Recharts dashboards	MANAGER
Settings	/settings + appearance, audit, features, organization, security	MANAGER
Student portal	/portal + appointments, documents, health (Saba), history	STUDENT

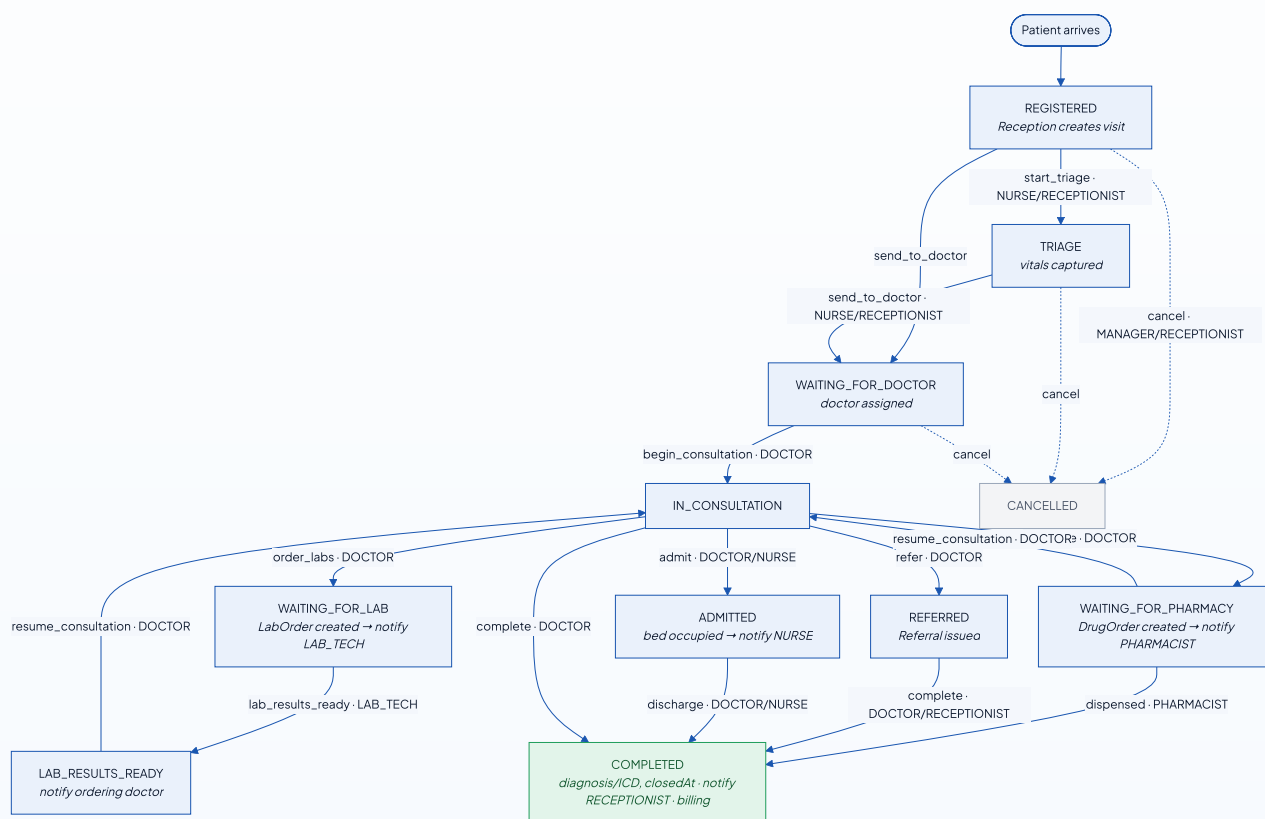
06 Clinical Workflow

The visit is the clinical spine. One machine carries a patient from registration through triage, consultation, lab, pharmacy, wards or referral, to completion — with lab results looping back to the doctor rather than closing the visit.

Machine: `src/server/fsm/machines/visit.ts`; orchestrator: `transitionVisit` in `src/server/services/visit.ts`. Each transition names the roles allowed to fire it; `resolveTransition()` throws unless both the state and the actor's role are valid. Every move is wrapped in a DB transaction, logged to `StateTransition`, and often fans out a notification.

FIG. 3 End-to-end clinical workflow with role lanes

Reception → triage → doctor → lab → results → pharmacy → wards / referral → billing → completed. Labels show the firing role; lab results route back to the doctor.



6.1 State-by-state, role-by-role

STEP	EVENT	FROM → TO	ROLE(S)	SIDE-EFFECTS
Registration	<i>(create)</i>	→ REGISTERED	RECEPTIONIST	Patient + visit created; optional SIMS lookup
Triage / vitals	start_triage	REGISTERED → TRIAGE	NURSE · RECEPTIONIST	Captures Vitals
Queue to doctor	send_to_doctor	→ WAITING_FOR_DOCTOR	NURSE · RECEPTIONIST	Assigns doctorId
Start consult	begin_consultation	→ IN_CONSULTATION	DOCTOR	Claims visit if unassigned
Order labs	order_labs	→ WAITING_FOR_LAB	DOCTOR	Creates LabOrder → notify LAB_TECH
Results back	lab_results_ready	→ LAB_RESULTS_READY	LAB_TECH	Notify ordering doctor
Re-assess	resume_consultation	→ IN_CONSULTATION	DOCTOR	Doctor reviews results
Prescribe	prescribe	→ WAITING_FOR_PHARMACY	DOCTOR	Creates DrugOrder → notify PHARMACIST
Dispense (close)	dispensed	→ COMPLETED	PHARMACIST	Auto-close on full dispense → notify RECEPTIONIST
Refer out	refer	→ REFERRED	DOCTOR	Creates Referral (ISSUED)
Admit	admit	→ ADMITTED	DOCTOR · NURSE	Creates Admission, occupies bed → notify NURSE
Discharge	discharge	→ COMPLETED	DOCTOR · NURSE	Frees bed; closes visit

STEP	EVENT	FROM → TO	ROLE(S)	SIDE-EFFECTS
Complete	<code>complete</code>	→ <code>COMPLETED</code>	DOCTOR	Writes diagnosis/ICD, sets <code>closedAt</code>
Cancel	<code>cancel</code>	early → <code>CANCELLED</code>	MANAGER · RECEPTIONIST	Records reason

THE LOOP THAT MATTERS

On `submit_results`, the lab technician records per-item values and flags and, if the visit is `WAITING_FOR_LAB`, flips it to `LAB_RESULTS_READY` and notifies the ordering doctor. Results route **back to the doctor** for re-assessment rather than closing the encounter — the clinical judgement stays with the physician.

6.2 Sub-machines feeding the spine

Lab order

`ORDERED` `COLLECTING` `IN_PROGRESS`

`RESULTS_READY` `COMPLETED`

LAB_TECH-driven (`collect`, `process`, `submit_results`, `complete`); `complete` also DOCTOR; `cancel` LAB_TECH/MANAGER. Per-item result flags: `NORMAL` `LOW/HIGH` `CRITICAL`.

Drug order

`ORDERED` `PARTIALLY_DISPENSED` `DISPENSED`

PHARMACIST-driven (`record_partial`, `dispense_all`, both form `dispense`; `cancel` PHARMACIST/MANAGER). Full dispense from the last stop auto-closes the visit.

07 Operational Workflows

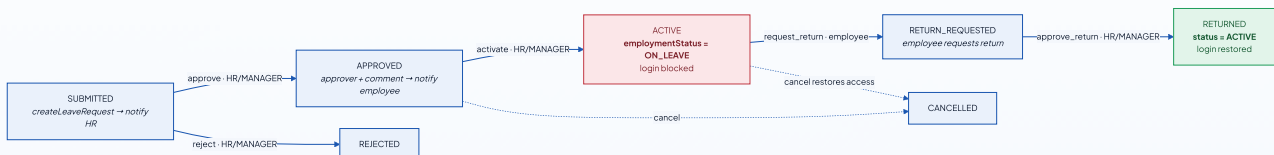
The same FSM discipline governs the back office — HR leave that couples directly to account access, store procurement from request to goods-receipt, and pharmacy dispensing that always draws from the earliest-expiring batch first.

7.1 HR leave — process coupled to security

Machine: `machines/leave.ts` ; service: `services/hr.ts` . Roles HR and MANAGER approve, reject, activate and approve return. The distinctive move: when a leave reaches **ACTIVE** , the employee's `employmentStatus` flips to `ON_LEAVE` , which **blocks their login** until they return — a clean example of process driving access control.

FIG. 4 HR leave flow with account-access coupling

Submitted → approved → active → return-requested → returned. Activation locks the login; return (or cancel/reject of an active leave) restores it.

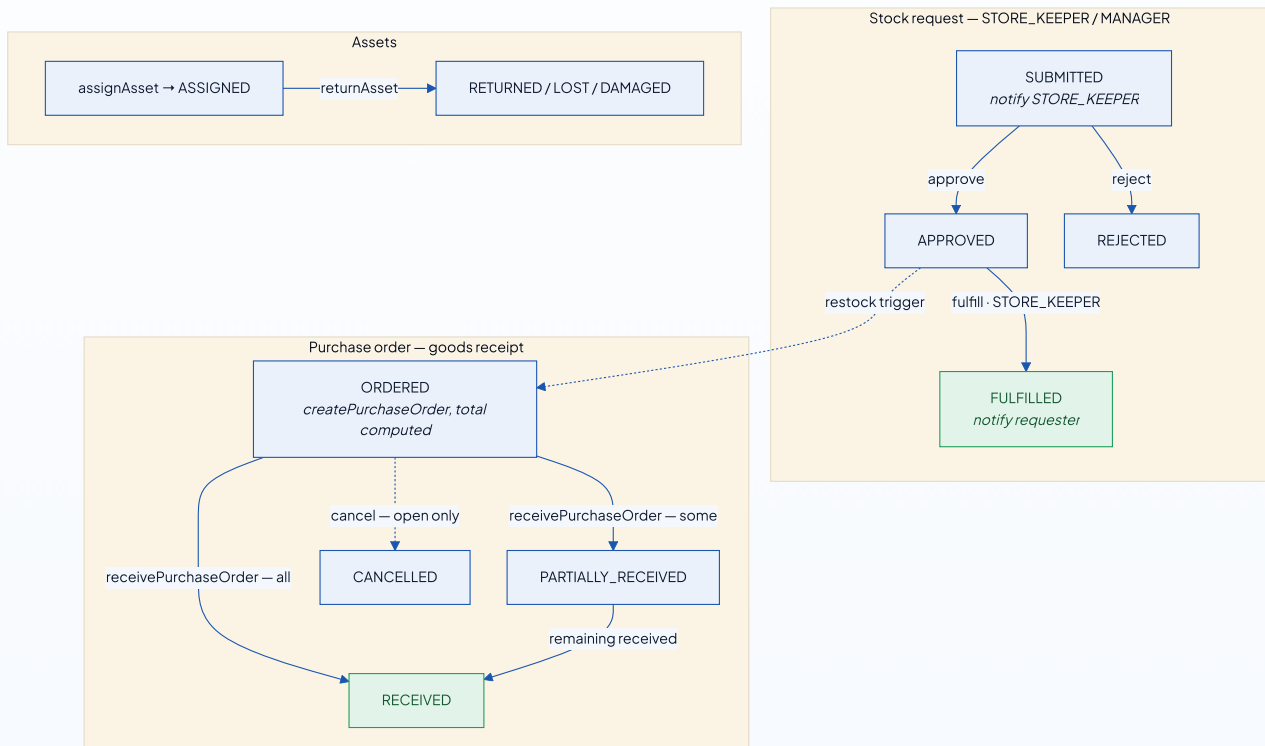


7.2 Store & procurement — request to goods—receipt

Service: `services/store.ts`. Two chains: a general **stock request** FSM, and the **purchase-order → goods-receipt** flow. `createStockRequest` notifies `STORE_KEEPER`; each decision notifies the requester back. Purchase orders compute a total on creation; `receivePurchaseOrder` increments each line's received quantity and moves the PO to **RECEIVED** (all in) or **PARTIALLY_RECEIVED**. Every step logs a `StateTransition` and an audit entry.

FIG. 5 Procurement / stock flow

Stock request (submitted → approved → fulfilled) and the parallel purchase-order chain (ordered → partially/fully received), with asset assignment as a sibling track.

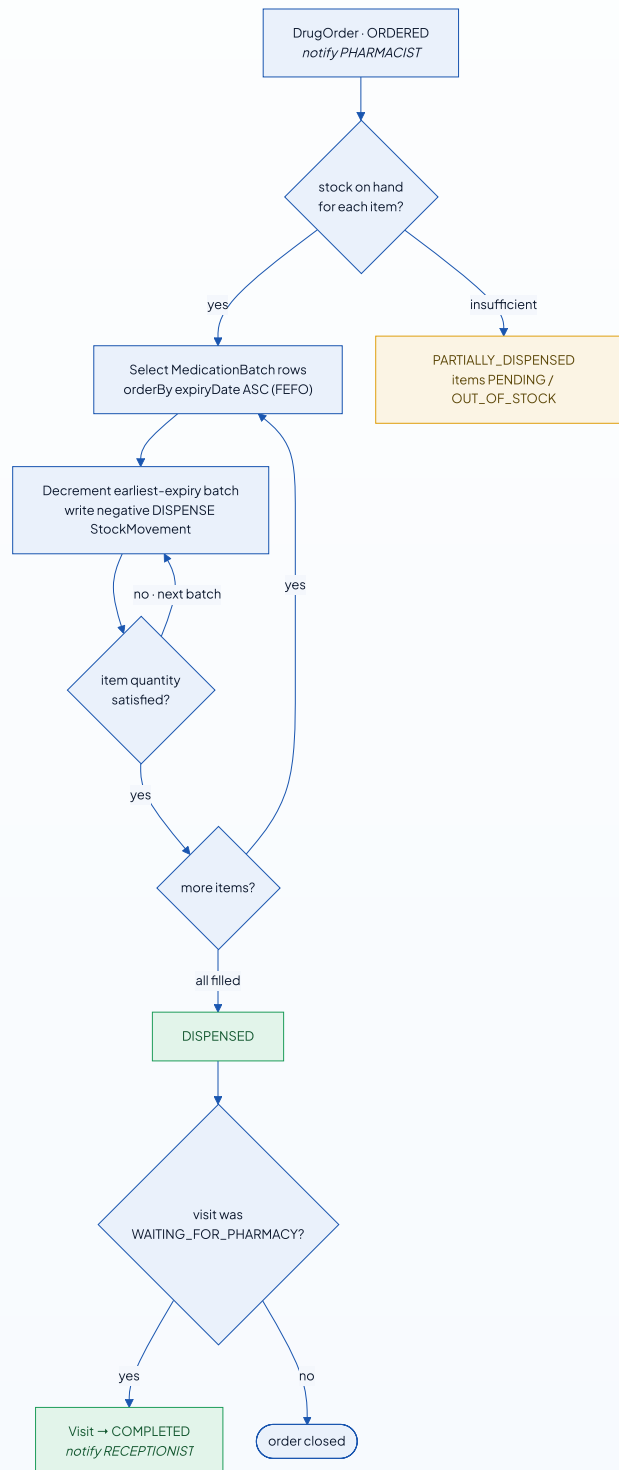


7.3 Pharmacy — FEFO dispensing

Service: `services/pharmacy.ts`. Stock is held in `MedicationBatch` rows (batch number, expiry date, quantity, cost/sell price, supplier). The critical rule is **FEFO — First-Expiry-First-Out**: `dispenseFromBatches()` decrements from batches **earliest-expiry-first** (`orderBy: { expiryDate: "asc" }`), writing a negative `DISPENSE` stock movement per batch for a complete audit trail. Partial fills mark items **PENDING** / **OUT_OF_STOCK**; full fills reach **DISPENSED**, and when the visit was `WAITING_FOR_PHARMACY` it flips to **COMPLETED** — pharmacy is the last stop.

FIG. 6 Pharmacy FEFO dispensing flow

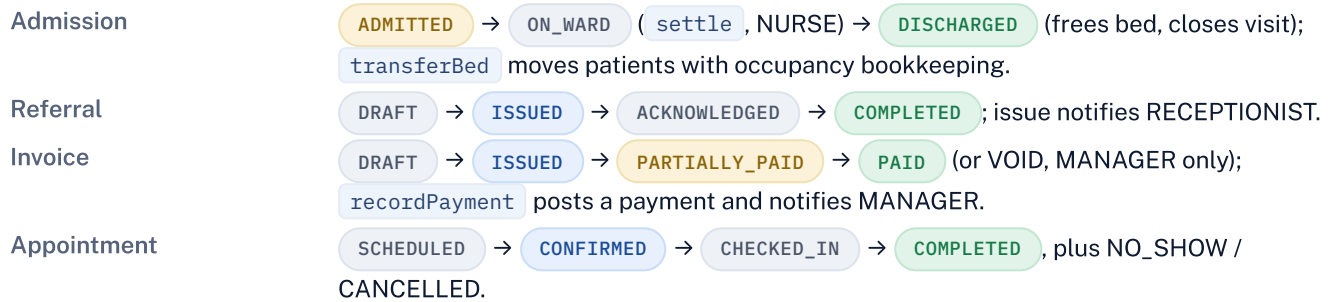
A drug order draws from medication batches ordered by earliest expiry, logging one movement per batch, until the order is fully or partially filled — a full fill from the last stop auto-completes the visit.



WHY FEFO

Dispensing the earliest-expiring stock first minimizes wastage from expiry and is the accepted good-practice for pharmacy inventory. Low-stock (on-hand $\leq$ reorder level), near-expiry (≤ 90 days) and expired batches are computed on demand and surfaced through management dashboards and Saba's operational context.

7.4 Other machines at a glance



08 Security & Compliance

Layered defence: role-based access control, dual Auth.js credential providers, multi-factor authentication with passkeys, an on-leave login lockout, a best-effort audit trail, edge middleware, and cryptographically verifiable documents.

8.1 Role-based access control

The `Role` enum defines eight roles. Session guards in `src/server/session.ts` (all `"server-only"`) provide `getActor()`, `requireStaff()`, `requireStudent()`, and `requireRole(...roles)` — the last **always lets MANAGER through**, else redirects non-matching roles to `/dashboard`. `SECTION_ACCESS` maps each top-level section to the roles allowed (or `null` = any authenticated staffer), and `canAccess()` drives both the sidebar and section guards.

Middleware route protection (`src/middleware.ts`) runs the edge-safe Auth.js config. Public paths are `/`, `/login`, `/student-login`, and `/verify[/...]`. Unauthenticated users are redirected to the correct login preserving `callbackUrl`; **students are confined to `/portal`** and staff hitting `/portal` are bounced to `/dashboard`.

8.2 Auth.js credential providers & MFA

Dual providers (`src/auth.ts`)

staff — email + password; looks up the lower-cased email, rejects inactive accounts, `bcrypt.compare` s the password. **Blocks any non-`ACTIVE` employment status** via `BlockedSignin`. If MFA is enabled, requires a valid second factor.

student — `studentId` + password against `Patient.portalPasswordHash`, gated on `portalEnabled`. Students get `role: null`, `kind: "student"`. Session strategy is JWT.

Multi-factor (`mfa.ts`, `webauthn.ts`)

- **TOTP** via `otpauth` — base32 secret, 6-digit/30s, ± 1 step drift, QR enrolment.
- **Passkeys (WebAuthn)** via `@simplewebauthn` — `rpID` / `origin` derived from request headers so it works on any host.
- **Single-use recovery codes** — 10 codes shown once, bcrypt-hashed, **consumed on use** (`usedAt` stamped).

ON-LEAVE LOGIN LOCKOUT

When a `LeaveRequest` reaches `ACTIVE`, the employee's `employmentStatus` becomes `ON_LEAVE`. The staff `authorize` flow rejects any non-`ACTIVE` status, so on-leave, suspended, and terminated staff cannot sign in. Process state and access control are the same fact.

8.3 Audit trail

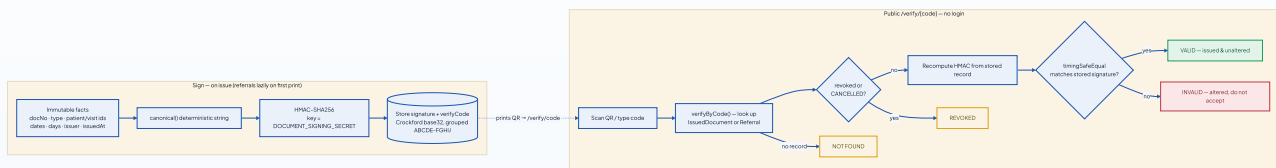
`audit(actorId, action, entityType?, entityId?, meta?)` writes to `AuditLog`. It is **best-effort**: a logging failure must never break the primary operation, and if the `actorId` foreign key can't be resolved (e.g. a session that predates a reseed) it retries as a system entry. Separately, `logTransition()` writes the FSM `StateTransition` trail, and `notifyRole` / `notifyUser` fan notification rows to a role or an individual.

8.4 Tamper-evident documents — HMAC sign & public verify

Every issued document and referral carries a short **public verify code** (printed as text and QR) and an **HMAC-SHA256 signature** over the record's *immutable* facts (`src/server/services/document-signing.ts`). The signing key is `DOCUMENT_SIGNING_SECRET` (falling back to `AUTH_SECRET` , then a dev default). Documents are signed on creation; referrals are signed **lazily on first print**. Verification recomputes the HMAC from the stored record and compares with `timingSafeEqual` — **a photoshopped certificate will not verify; a genuine one always will.**

FIG. 7 HMAC document sign & verify

Signing HMACs a canonical string of immutable facts and stores it with a verify code; the public /verify route recomputes and timing-safe-compares, yielding valid / invalid / revoked / not-found.



The public verify page renders one of four states, each with a distinct icon and tone: **valid** "issued by the clinic and has not been altered", **invalid** "details don't match — may have been altered, do not accept", **revoked** , and **not found** . Verification trusts the server's recomputation, not anything printed on the page.

09 Saba — the AI Assistant

One assistant with two faces — a clinical copilot for doctors and a caring health guide for students — built on a provider-agnostic engine and wrapped in a deliberately strict, read-only guardrail spine.

Saba (`src/server/ai/saba.ts` , `src/app/api/saba/route.ts` , `src/components/saba/*`) is gated by the `ai.assistant` flag. It talks to **any OpenAI-compatible** `/chat/completions` endpoint, so it runs on Groq, OpenAI, Ollama, or a self-hosted gateway with no code change — purely env-configured. `SABA_PROVIDER` selects per-provider defaults (groq → `openai/gpt-oss-120b` in the demo config; openai → `gpt-4o-mini` ; ollama → `llama3.1`), overridable by `SABA_BASE_URL` , `SABA_API_KEY` , `SABA_MODEL` . Ollama needs no key, so Saba can run fully on-prem with no data leaving the clinic. Responses stream: `streamSaba()` is an async generator that parses SSE `data:` lines and yields token chunks the browser appends live.

A · Doctor / staff copilot

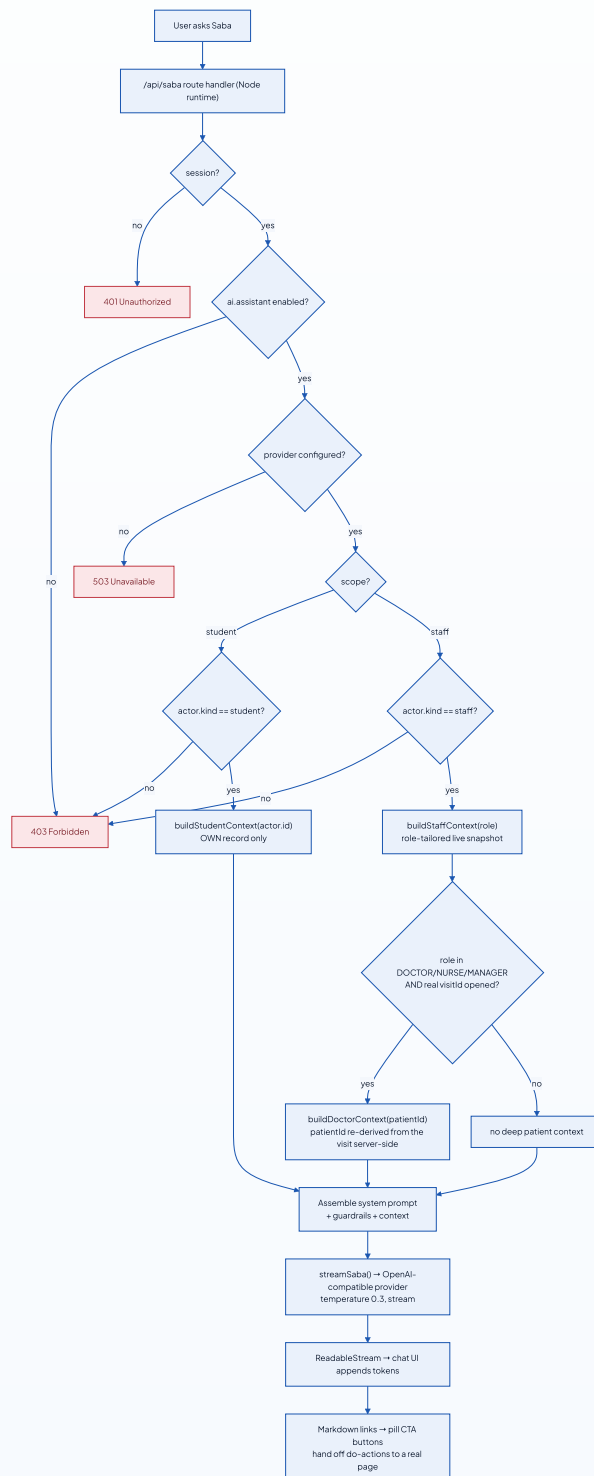
Scope `staff` , mounted app-wide as a floating widget when `ai.assistant` is on. History-aware (last 12 turns) and role-aware, with live read-only tooling per role. For a doctor on a real open visit it becomes a clinical copilot: history-aware differentials, tests to consider, red-flag flagging — framed as **decision support**.

B · Student health guide

Scope `student` , in the portal — "a caring health guide ... like a kind older sister." Uses the student's own past visits only, to give gentle self-care advice and nudge them to visit the clinic or seek emergency care when appropriate. Explicitly not a doctor.

FIG. 8 Saba request flow — auth, authorization, context, stream, CTA

User → session → feature gate → role/scope authorization → server-built read-only context → provider stream → CTA links rendered as pill buttons.



9.1 Guardrails — the safety spine

Saba's constraints are enforced in **both** the system prompt and the route. Four layers:

1 Read-only

Saba is **READ-ONLY**: it can look up and explain but *cannot make any change, approve, order, dispense, book or edit anything*, and never claims it did. It has no tools that mutate — it only reads context the server pre-builds. To *do* something, it emits a markdown link to the exact page, rendered as a pill-shaped **CTA button**.

2 Scope guard

STAY ON TASK — Saba only helps with this clinic. Asked anything unrelated (trivia, coding, maths, essays, jokes, world facts, chit-chat) it declines in one short line. The student persona gently declines and offers health help instead.

3 Authorization

Students: must be `kind === "student"`; context is built from `actor.id` only — *own record only*.
Staff: deep clinical patient context loads *only* for `DOCTOR/NURSE/MANAGER` and *only* for a real `visitId` the user opened; the server re-derives `patientId` from the visit, never from a client-supplied id. This closes the "ask about an arbitrary MRN" hole.

4 Clinical-safety framing

`SAFETY_DOCTOR` : decision *support*, never a decision maker; no definitive diagnosis; differentials and tests framed as considerations; flag red flags; note clinical correlation. `SAFETY_STUDENT` : a friendly guide, *not* a doctor — do not diagnose or prescribe. The chat header carries an "AI · verify advice" badge.

THE AUTHORIZATION GATE IS REAL, NOT JUST PROMPT TEXT

Route-level enforcement (`route.ts`) is the true gate: it returns 401 without a session, 403 if the flag is off or the persona/role mismatches, and 503 if no provider is configured. Because Saba is read-only and re-derives patient identity server-side, the worst-case blast radius of a prompt-injection attempt is bounded to information the actor was already authorized to see.

9.2 Role-tailored live context

`buildStaffContext({id, role})` assembles a role-specific "what needs my attention" snapshot — all read-only, all server-side:

ROLE	LIVE SNAPSHOT SABA CAN ANSWER FROM
MANAGER	In clinic now / completed today; who's on leave + pending count; stock lows, batches expiring ≤90d, expired; open POs + pending stock requests.
PHARMACIST	Prescriptions awaiting dispensing; low-stock list; expiry counts (≤90d / expired).
LAB_TECH	Worklist counts by state (ordered / collecting / in-progress / results ready).

ROLE	LIVE SNAPSHOT SABA CAN ANSWER FROM
HR	Pending leave requests (names); who's currently on leave; staff present today.
STORE_KEEPER	Pending stock requests (item × qty); open POs; low-medicine count.
NURSE	Patients waiting for a doctor; inpatients on ward; bed occupancy.
RECEPTIONIST	Registered today; waiting for a doctor now.
DOCTOR	Your open patients right now; plus deep <code>buildDoctorContext</code> "chart on a page" for the open visit.

10 Documents the System Generates

Six document types, each an immutable snapshot printed on a white-label letterhead and sealed with a scannable verify code. Below, a faithful recreation of the sick-leave certificate the platform produces.

The `DocumentType` enum covers `SICK_LEAVE`, `PRESCRIPTION`, `REFERRAL`, `LAB_REPORT`, `ADMISSION_SUMMARY`, `VISIT_SUMMARY`, each with a print route. Every issued document is an immutable `IssuedDocument` row with a `docNo` (e.g. `DOC-2026-00042`) and a `payload` **JSON snapshot** of exactly what was printed, so re-prints reproduce the original even if the underlying record later changes. The shared `Letterhead` reads from `getBranding()`, so white-label branding flows into every printout.

**Dire Dawa University**

Student Clinic Center

Medical Certificate — Sick Leave

OFFICIAL
MEDICAL DOCUMENT**MEDICAL CERTIFICATE**

CERTIFICATE OF SICK LEAVE

PATIENT

Meron Tadesse

MRN

CLN-004182

AGE / SEX

21 / Female

VISIT

V-2026-01947

This is to certify that **Meron Tadesse** was examined at this facility and, on medical grounds, is advised to rest from **18 Aug 2026** to **22 Aug 2026** (5 days).

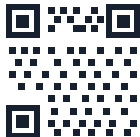
Diagnosis: Acute viral pharyngitis (ICD-10 J02.9).

Recommendation: Rest, oral fluids, and paracetamol as needed. Return to the clinic if fever persists beyond 72 hours or breathing difficulty develops.

OFFICIAL
STAMP**Dr. Samuel Bekele**

General Practitioner

ATTENDING PHYSICIAN



SCAN TO VERIFY

Authentic document**3J3HD - VKSCV**<https://clinic.ddu.edu.et/verify/3J3HD-VKSCV>

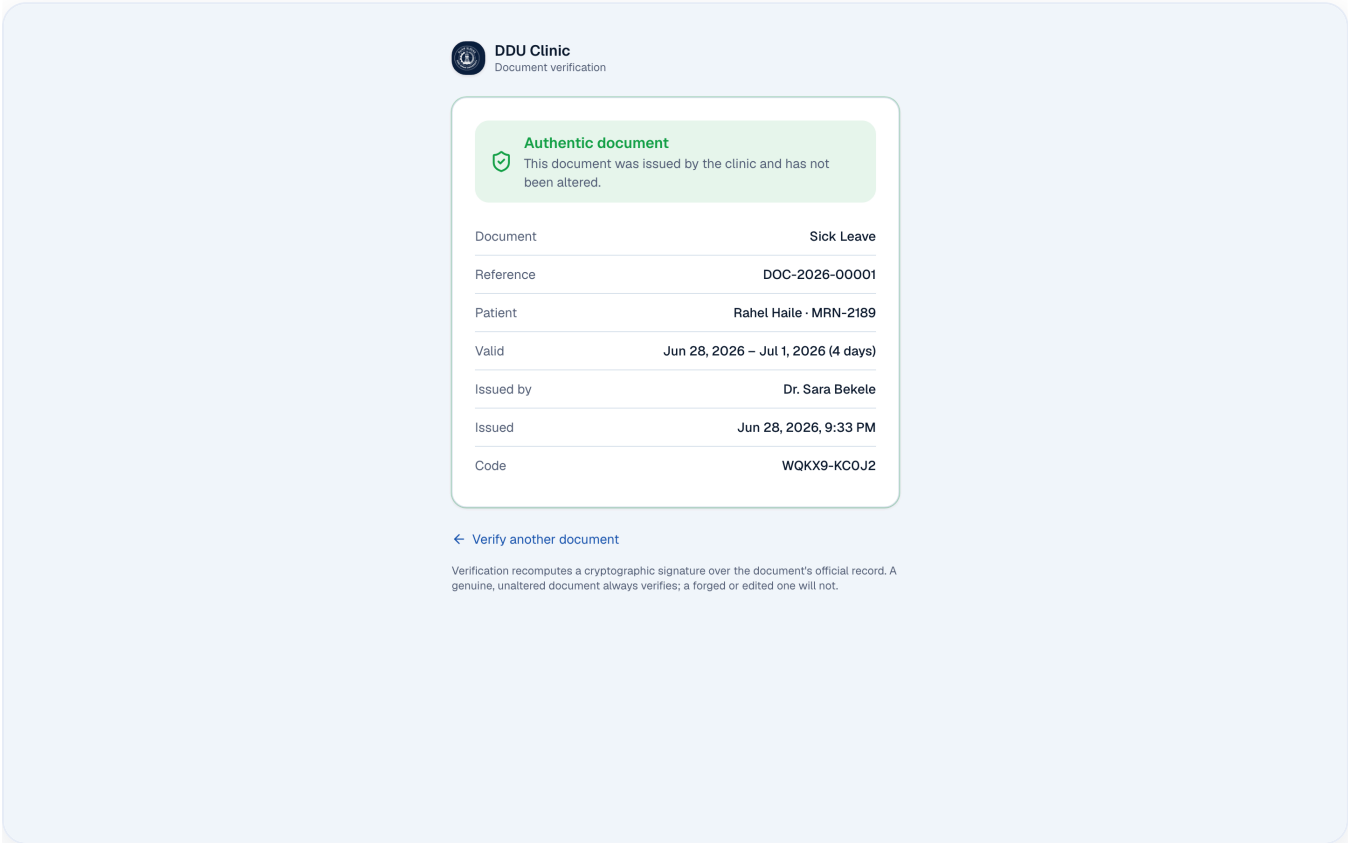
Ref: DOC-2026-00042

Issued: 22 Aug 2026

Faithful recreation of `src/app/print/sick-leave/[docId]/page.tsx` with the shared `Letterhead`, `Field`, `SignatureLine`, and `VerifySeal` components. The QR grid is a stylized placeholder; the live seal embeds a real QR pointing at `/verify/<code>`.

10.1 The verification counterpart

The QR and code on every document point at the **public** `/verify` page — no login needed. Anyone (a faculty member, an employer) can confirm a certificate's authenticity in seconds. The page recomputes the cryptographic signature over the official record; a genuine, unaltered document always verifies, a forged one never does.



The public `/verify/[code]` result page — recomputing the HMAC over the clinic's official record to confirm a document is issued and unaltered.

10.2 The rest of the document family

Alongside sick-leave, the platform issues signed printables for visit summaries, referral letters, and discharge summaries. The discharge summary is a client component with **"Include sections:" checkboxes** so the clinician chooses which blocks appear before printing, and computes length-of-stay. All are signed and sealed.



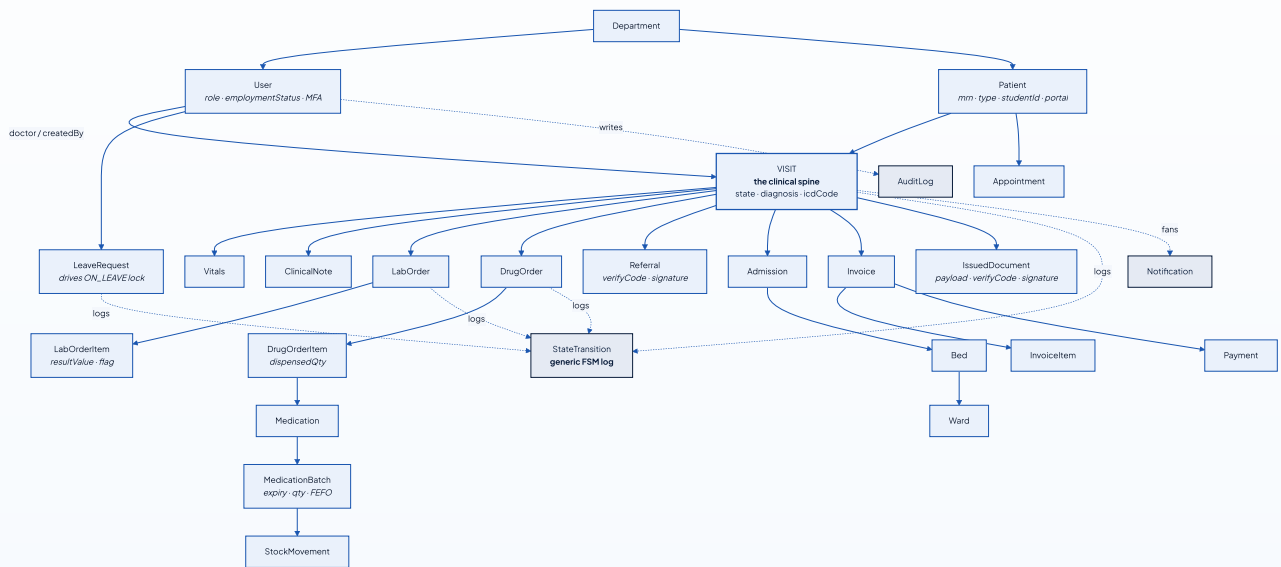
Concise previews of the visit-summary, referral-letter and discharge-summary printables (`src/app/print/{visit-summary,referral,discharge}/*`). Each embeds a verify seal identical in mechanism to the sick-leave certificate above.

11 Data Model

The full schema is 1,185 lines of Prisma. At its heart is the Visit — the encounter the FSM drives — surrounded by identity, clinical detail, pharmacy stock, billing, and the cross-cutting platform tables.

FIG. 9 Key entities & relationships

The Visit is the hub of the clinical spine; Patient and User anchor identity; StateTransition and AuditLog are the cross-cutting event logs every entity feeds.



11.1 Entity groups

Identity & RBAC

User — staff account, hub of ~30 relations;
RecoveryCode (hashed, single-use); **Authenticator** (WebAuthn passkey); **Department**.

Patients

Patient — MRN, type
 (STUDENT/STAFF/DEPENDENT/EXTERNAL), source (MANUAL/SIMS), portal login, demographics; owns visits, documents, appointments, invoices, allergies.

Clinical spine

Visit (FSM-driven encounter) with **Vitals**, **ClinicalNote**, lab/drug orders, referrals, one admission, documents, invoices.

Pharmacy

Medication → **MedicationBatch** (lot/expiry/qty) → **StockMovement** (RECEIVE/DISPENSE/ADJUST/EXPIRE/RETURN);
DrugOrder → **DrugOrderItem**.

Wards, billing, HR

Ward → **Bed** → **Admission**; **Invoice** → **Invoiceltem** + **Payment**; **LeaveRequest**, **Attendance**, **Shift**, **LeaveBalance**.

Cross-cutting

IssuedDocument (payload + signature), **Notification** (recipient and/or role), **StateTransition** (generic FSM log), **AuditLog**, **FeatureFlag**, **Setting**.

12 Deployment & Operations

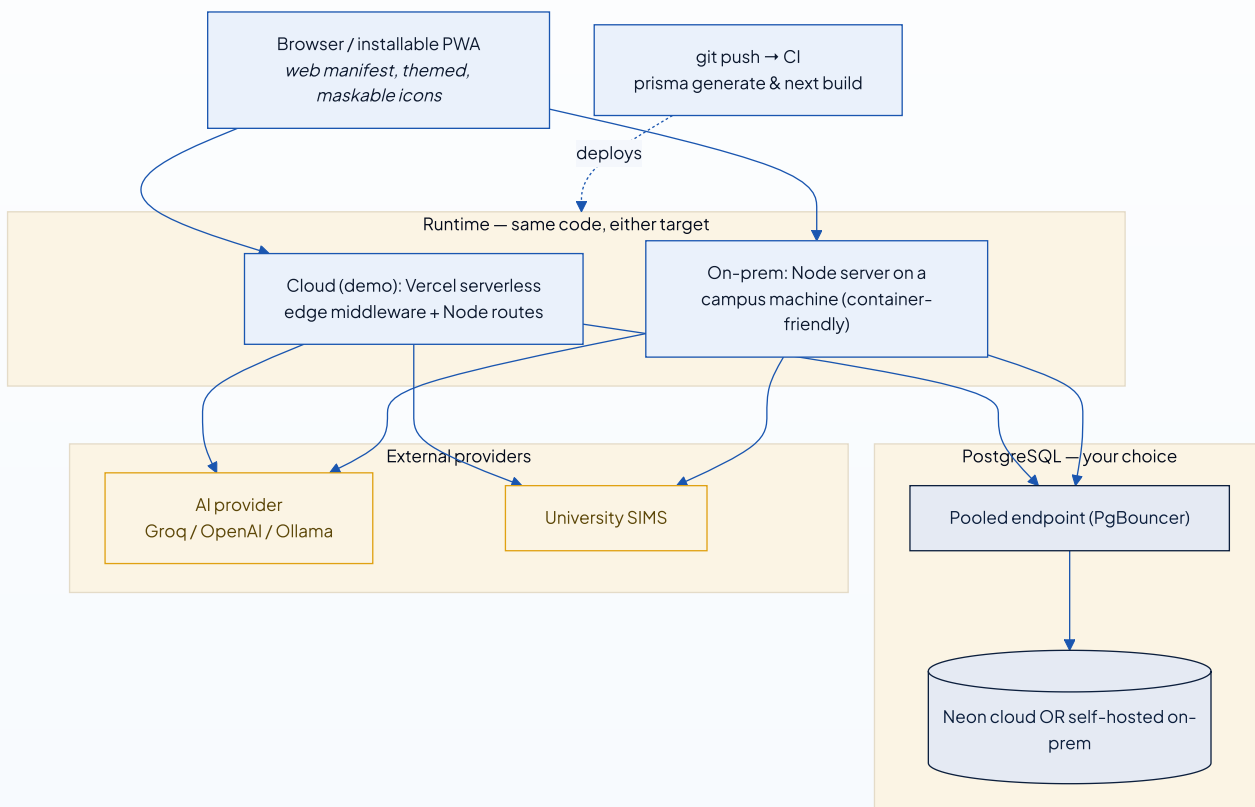
Cloud-native and highly scalable, and — crucially — **deployment-portable**. The public demo runs on Vercel + Neon, but the exact same codebase deploys **on-premises on a campus server with no data loss and no vendor lock-in**. Configured entirely by environment, shipped by a git push, and installable as a PWA.

DEMO VS. PRODUCTION

The live demo is hosted on **Vercel** for convenience — but that is **not** a requirement. DDU Clinic is a standard Next.js + PostgreSQL application, so production can run **fully on-premises inside the university** (a campus server or the DDU data centre) with the database and every record staying on university infrastructure. Because it is env-configured and container-friendly, moving from the cloud demo to on-prem is a configuration change, **not** a rewrite — and carries **no data loss**. Cloud-native by design, portable by principle.

FIG. 10 Deployment topology — cloud or on-prem, same code

The same application image runs as cloud serverless functions (demo: Vercel) or as a Node process on a campus server; PostgreSQL is Neon (cloud) or self-hosted on university infrastructure. AI can even run on-prem via Ollama, so no clinical data need leave the campus.



Cloud-native, runs anywhere

A standard Next.js App Router app. The demo runs as Vercel serverless functions; the identical build runs as a plain **Node process on a campus server** (container-friendly). Auth is edge-split: `auth.config.ts` (edge-safe) powers middleware; the full `auth.ts` runs on Node. **No cloud-only APIs are required.**

PostgreSQL — cloud or on-prem

`datasource db` is plain `postgresql` via `url = env("DATABASE_URL")`. Point it at Neon (cloud demo, pooled PgBouncer) **or a PostgreSQL server inside the university** — every record then stays on campus infrastructure. Switching is one env var; no schema or code change.

Env-based config

All environment-specific behavior is env-driven: `DATABASE_URL`, `AUTH_SECRET`, `AUTH_TRUST_HOST`, `DOCUMENT_SIGNING_SECRET`, the `SIMS_*` integration, and the `SABA_*` AI knobs. Auth.js derives the URL from the request, so redirects follow whatever host/port you run on.

One build, any target

`npm run build = prisma generate && next build`, regenerating the Prisma client against the schema every time. In the cloud a git push triggers CI + rollout; on-prem the same command builds on the campus server (or in a container) — identical artifact either way.

PWA — HONEST STATUS

The app is **installable**: `public/manifest.webmanifest` (standalone display, themed, maskable icons) is wired via `metadata.manifest` and `appleWebApp`. However, on this branch there is **no service worker and no Serwist wiring** — so today the PWA is manifest-based (installable, themed), *not* a fully offline-caching service worker. That is the intended direction (Serwist is already a dependency) but not yet active.

12.1 Scaling posture

Because functions are stateless and short-lived, horizontal scale is handled by the platform; the bounded resource is database connections, which the Neon pooled endpoint absorbs. The audit and transition logs are append-only and keyed by entity, making them a natural feed for an external warehouse without touching the hot path.

13 Roadmap & Future Ecosystem

Every future connector is a natural extension of foundations already in place — the pluggable-adapter pattern, the FSM engine, and signed documents. The following are clearly aspirational.

ALREADY SHIPPING

SIMS student lookup — a pluggable `SimsProvider` adapter with mock and HTTP implementations, env-selected, gated by `clinical.sims_lookup`. **AI providers** — the same pluggable pattern over OpenAI-compatible endpoints, on-prem-capable via Ollama. **White-label branding** — `getBranding()` injects org name, app name and theme colours into the live layout and every printout.

13.1 Future connectors FUTURE

Deeper SIMS / registrar

Beyond lookup: write-back of clinic-issued sick-leave to the registrar, enrolment/eligibility checks, automatic student-status sync. The `SimsProvider` interface is the seam.

National HMIS / DHIS2

Export aggregate indicators (visits, ICD-10 diagnoses, immunizations) to the national health-information exchange; the structured coded data already exists to feed it.

Insurance & payments

Plug billing into insurance eligibility + claims and mobile-money / telebirr-style gateways; the invoice FSM and Payment records are the integration points.

Telemedicine

A remote-consultation channel layered onto the visit spine (a virtual `IN_CONSULTATION`), reusing the same ordering / prescribing / referral machinery.

Real-time broadcasting

Replace the 25-second notification polling with websockets / SSE / pub-sub so notifications, queues and ward boards update instantly across stations.

Analytics / BI exports

The StateTransition + AuditLog + domain tables are a ready warehouse feed for throughput, turnaround-time and utilization dashboards beyond the built-in reports.

Verification as a trust service

Extend the `/verify` HMAC scheme to third-party verifiers (employers, faculty) and to signed QR on any future document type.

Mobile / tablet & student status app

Native mobile and tablet clients, and a lightweight student app to check clinic status and queue position — building on the installable PWA foundation.

13.2 Toward a campus digital ecosystem

Each module already exposes a clean seam for campus-wide integration: **SIMS** ties the clinic to the student information system; **signed documents** + `/verify` give faculty and employers a trust endpoint; **HR leave** ↔ `employmentStatus` **lockout** models the access-control pattern a campus IAM could federate; **billing** is the hook for campus finance and insurance; and **feature flags** + **white-label branding** let the same platform serve

other university health centres as a multi-tenant service. Because of the FSM engine, any new campus workflow is added as one declarative machine — with role-gated transitions, audit, and notifications for free.

14 Appendix

The technology stack, demo accounts for evaluation, and a short glossary of the terms used throughout this document.

14.1 Technology stack

CONCERN	CHOICE	VERSION
Framework	Next.js (App Router)	next 16.3.1
UI runtime	React 19 (Server Components + Actions)	react 19.2.8
Language	TypeScript	typescript ^5
ORM	Prisma	^6.19.3
Database	PostgreSQL (Neon serverless in prod)	—
State machines	XState v5 + bespoke declarative engine	xstate ^5.32.5
Auth	Auth.js / NextAuth v5 (beta)	next-auth ^5.0.0-beta.32
Passwords	bcryptjs	^3.0.3
MFA — TOTP	otpauth + qrcode	otpauth ^9.5.1
MFA — Passkeys	@simplewebauthn (server + browser)	^9.0.3
Styling	Tailwind CSS v4	^4
PWA	Servist (dependency; not yet wired)	^9.5.12
Charts	Recharts	^3.10.1
Forms / validation	react-hook-form + Zod	RHF ^7.85 · zod ^4.4
Client data	@tanstack/react-query	^5.101.4
UI primitives	Radix UI · lucide-react · sonner · CVA	—
AI copilot	Groq / OpenAI-compatible (plain fetch)	—
Tests	Vitest + Testing Library + jsdom	vitest ^4.1.11

14.2 Demo accounts

EVALUATION LOGINS

Staff sign in at `/login` with email + password. Pattern: `<role>@clinic.test` / `password` .

manager@clinic.test

doctor@clinic.test

nurse@clinic.test

labtech@clinic.test

pharmacist@clinic.test

reception@clinic.test

hr@clinic.test

store@clinic.test

Student signs in at `/student-login` : student ID `DDU/1001/14` / password `student` . All demo passwords are for evaluation only.

14.3 Glossary

Affordance	A serializable projection of a legal, role-permitted transition — what the client renders as an action button.
FSM engine	The declarative finite-state-machine engine that is the single authority for legal transitions and role authorization.
HATEOAS-style	The backend-driven-UI contract: the server tells the client which actions are available; the client renders exactly those.
FEFO	First-Expiry-First-Out — dispensing pharmacy stock from the earliest-expiring batch first to minimize wastage.
HMAC	Keyed hash (SHA-256) computed over a document's immutable facts to make forgery detectable at <code>/verify</code> .
Feature flag	A registry-defined toggle (TENANT / RELEASE / OPS / PERMISSION / EXPERIMENT) gating a module, action, or UI branch.
Modular monolith	A single deployable application organised into clean service modules with boundaries drawn for later decomposition.
RBAC	Role-based access control across eight staff roles; MANAGER always passes role guards.
SIMS	The university Student Information Management System, reached through a pluggable lookup adapter.
StateTransition	The generic, platform-wide FSM audit log every entity move is written to.



END OF DOCUMENT

DDU Clinic — Platform Architecture & Product Documentation

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